

WHAT'S MY ACE SCORE?

Prior to your 18th birthday:

1. Did a parent or other adult in the household **often or very often** . . .
Swear at you, insult you, put you down, or humiliate you?

or
Act in a way that made you afraid that you might be physically hurt?
Yes No If yes, enter 1 _____
2. Did a parent or other adult in the household **often or very often** . . .
Push, grab, slap, or throw something at you?

or
Ever hit you so hard that you had marks or were injured?
Yes No If yes, enter 1 _____
3. Did an adult or person at least 5 years older than you **ever** . . .
Touch or fondle you or have you touch their body in a sexual way?

or
Attempt or actually have oral, anal, or vaginal intercourse with you?
Yes No If yes, enter 1 _____
4. Did you **often or very often** feel that . . .
No one in your family loved you or thought you were important or special?

or
Your family didn't look out for each other, feel close to each other, or support each other?
Yes No If yes, enter 1 _____
5. Did you **often or very often** feel that . . .
You didn't have enough to eat, had to wear dirty clothes, and had no one to protect you?

or
Your parents were too drunk or high to take care of you or take you to the doctor if you needed it?
Yes No If yes, enter 1 _____
6. Was a biological parent **ever** lost to you through divorce, abandonment, or other reason?
Yes No If yes, enter 1 _____

7. Was your mother or stepmother:

Often or very often pushed, grabbed, slapped, or had something thrown at her?

or

Sometimes, often, or very often kicked, bitten, hit with a fist, or hit with something hard?

or

Ever repeatedly hit over at least a few minutes or threatened with a gun or knife?

Yes No If yes, enter 1 _____

8. Did you live with anyone who was a problem drinker or alcoholic, or who used street drugs?

Yes No If yes, enter 1 _____

9. Was a household member depressed or mentally ill, or did a household member attempt suicide?

Yes No If yes, enter 1 _____

10. Did a household member go to prison?

Yes No If yes, enter 1 _____

Now add up your "Yes" answers: _____ This is your ACE Score.

EMDR THERAPY SCREENING AND DATA CHECKLIST

Name: _____ Date: _____

Type and amount of previous counseling: _____

Present medications: _____

☐ Results of objective tests _____

☐ Mental status exam plus dissociative disorder screening

☐ Sufficient rapport (feeling of safety, degree of truth telling)

☐ Ability to use self-control techniques tested positive with _____

☐ Personal and environmental stability: Factors needing attention are _____

☐ Life supports: _____

☐ Physical health (areas of concern include neurological impairment; pregnancy; cardiac, respiratory, and geriatric issues; seizures, and eye problems): _____

☐ Inpatient treatment (medical needs; nature of the traumatic memory; is client a danger to self or others?): _____

☐ Medication needs: _____

☐ Drug or alcohol abuse (supports; program; was briefing given of potential exacerbation?): _____

☐ Legal restraints (processing effects/forensic issues): _____

☐ Systems control: _____

☐ Secondary gains (action planned): _____

☐ Timing considerations: _____

☐ Reporting requirements: _____

- ☐ Terms in which theory was explained: information stored in brain, blocked learning, REM, hemispheres, or other? _____
- ☐ Informed consent: Mention possibilities that distressing, unresolved memories may surface; that unanticipated reactions may include disturbing emotions or sensations; and that between-session disturbance, such as nightmares or other memories, may occur.
- ☐ Test eye movement (client's preferences? which movements to avoid? alternative stimuli to be used?): _____
- ☐ Metaphors (train, tiger, tunnel, other?): _____
- ☐ Safe/Calm Place (specify place and word): _____
- ☐ Stop signal: _____
- ☐ Other: _____

CLIENT HISTORY FORM

Name: _____ Date: _____

1. What are the reasons the client came for therapy?

2. What are the client's goals?

3. What are the client's symptoms?

4. When did the symptoms start?

5. What else was happening at that time? (*contributing events*)

6. Have the symptoms changed? If so—how/when?

7. Why did the client decide to come in now?

8. What other situations may be contributing at this time?
9. Is there any crisis or situation needing an action plan? *(e.g., danger, family reunions, tests)*
10. Present medications *(plus effects of and feelings about them)*
11. Previous therapy:
 - Reason and focus
 - What kinds? *(descriptions of memorable interactions)*
 - Length of therapy
 - Quality of relationship with therapist *(any problems?)*
 - What characteristics does client look for in a therapist?
 - Why did the client stop treatment?
 - Results of therapy including:
 - What did s/he learn that was useful? How was it disappointing?
 - Was there anything s/he never addressed? *(events/situations/symptoms/issues)*
 - What did client like and not like in previous therapy?

12. Present relationships—spouse/partner/significant other/children (*include quality of these relationships*)
13. Other current caring friends and relatives
14. Current and previous work/school situations and relationships (*bosses/colleagues*)
15. Successes/Strengths/Feeling protective of anyone? (*useful for Cognitive Interweaves*)
16. How would client know if therapy is successful?
17. What would happen if therapy is successful? Would there be a downside?
Would anyone in the client's life have a problem with this?
18. Attachment issues?
19. Issues related to race, ethnicity, culture, and nationality?

20. Religious influences/issues
21. Issues related to gender identity and sexual preference
22. Relationships with parents—past and present (*use pictures from childhood to discuss, if possible*)
23. Quality/examples of relationship between parents—past and present
24. Quality/examples of relationships with siblings—past and present
25. Friends/mentors in childhood and adolescence (*“Who really cared about you? How did that feel?” Subsequent disappointments.*)
26. School experience/teachers/peers—positive and negative
27. “Is there anything I haven’t asked that you feel is important for me to know?”
28. Use of various self-control techniques in order to increase access to resources (*positive memories and projective experiences*)

29. How does client presently self-soothe? (*including exercise, yoga, meditation, drugs, drinking, shopping, etc.*)

30. Hobbies and fun activities:

31. Additions:

- Safe/Calm Place(s): image, emotions, sensations, cue word/phrase
- Resource Enhancement (e.g., RDI): image, emotions, sensations, cue word/phrase
- Additional: Light Stream, Breath, Hypnosis, Other (*specify*) _____
- Therapeutic relationship experiences/exercises (for stability within and between sessions)

32. Time line. Ask for 10 most disturbing memories and place them on a chronological time line. Explain 0–10 SUD scale and indicate level of disturbance as a baseline. Do the same for 10 most positive memories.

- Sometimes it is useful to ask: “On this time line, what are the most important events—good and bad—that formed the person you are today?” Or “When are the times that things changed?” Specifically ask for deaths/losses (including animals) and humiliations.

33. Negative Cognition checklist: Ask clients to check off the ones that give them a feeling in their body and/or “feels like them” when they are disturbed. These can be used to identify the earliest memories that set the groundwork for them. These events are also placed on the time line.

34. People and present situations that cause disturbance. These triggers are used to access specific memories of present events that will be targeted with an Affect Scan or Floatback to identify the pertinent Touchstone Memories. These Touchstone Memories will be processed in the first stage of the Three-Pronged Protocol. Then the triggers will be targeted and processed. Each triggering situation will also be the basis for the processing of a future template.
35. Observations of therapeutic, family, and other systemic interactions that indicate automatic responses and deficits that will need to be addressed.
36. List kinds of skills and experiences necessary to bring the client to a full level of mental health and adaptive functioning (happiness, confidence, bonding, contributing). For instance, what relationship skills or experiences are needed? Most needs will be further revealed as therapy progresses. What types of experiences are needed to maintain the client's motivation to continue therapy?
37. Personal feelings about client. To foster self-awareness, evaluate as if from a third-person perspective: What sensations arise in the therapist's body? How comfortable is the therapist with the way the client demonstrates affect? What personal issues may arise from client's problems? These involve the therapist's own memory networks that are stimulated. What may need to be processed? How comfortable does the therapist feel about disclosure/honesty.

WEEKLY LOG (TICES) REPORT

Date	Trigger	Image	Cognition	Emotion	Sensation/SUD

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EXAMPLES OF NEGATIVE AND POSITIVE COGNITIONS

Negative Cognitions	Positive Cognitions
Responsibility/Defectiveness	
I'm not good enough	I am good enough/fine as I am
I don't deserve love	I deserve love; I can have love
I am a bad person	I am a good (loving) person
I am incompetent	I am competent
I am worthless/inadequate	I am worthy; I am worthwhile
I am shameful	I am honorable
I am not lovable	I am lovable
I deserve only bad things	I deserve good things
I am permanently damaged	I am/can be healthy
I am ugly/my body is hateful	I am fine/attractive/lovable
I do not deserve . . .	I can have/deserve . . .
I am stupid/not smart enough	I am intelligent/able to learn
I am insignificant/unimportant	I am significant/important
I am a disappointment	I am OK just the way I am
I deserve to die	I deserve to live
I deserve to be miserable	I deserve to be happy
I am different/don't belong	I am OK as I am
I have to be perfect (out of inadequacy)	I am fine the way I am
Responsibility: Action	
I should have done something*	I did the best I could
I did something wrong*	I learned/can learn from it
I should have known better*	I do the best I can/I can learn
*What does this say about you? (e.g., I am shameful/I am stupid/I am a bad person)	I'm fine as I am
I am inadequate/weak	I am adequate/strong

(continued)

Negative Cognitions

Positive Cognitions

Safety/Vulnerability

I cannot trust anyone
I cannot protect myself
I am in danger
I am not safe
I am going to die
It's not OK (safe) to feel/show my emotions

I can choose whom to trust
I can learn to protect myself
It's over; I am safe now
I am safe now
I am safe now
I can safely feel/show my emotions

Power/Control/Choice

I am not in control
I am powerless/helpless
I cannot get what I want
I cannot stand up for myself
I cannot let it out
I cannot be trusted
I cannot trust myself
I cannot trust my judgment
I cannot succeed
I have to be perfect
I can't handle it

I am now in control
I now have choices
I can get what I want
I can make my needs known
I can choose to let it out
I can be trusted
I can/learn to trust myself
I can trust my judgment
I can succeed
I can be myself/make mistakes
I can handle it

EMDR FIDELITY RATING SCALE (EFRS)

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The EMDR Fidelity Rating Scale (EFRS; Korn, Maxfield, Smyth, & Stickgold, 2017) evaluates adherence to EMDR therapy's standard eight-phase treatment approach and three-pronged protocol (Shapiro, 2001). Originally developed for use in a study by van der Kolk et al. (2007), the scale was revised in 2016 after receiving feedback from researchers and raters who had used the scale. The EFRS is a comprehensive rating instrument designed to assess treatment integrity in a single EMDR session and across multiple sessions. The scale provides a breakdown of treatment components, with indicators for "acceptable" adherence, a rating system, and an easy-to-use scoring calculator. Fidelity ratings range from 0 ("no adherence") to 3 ("very good adherence"), with a cutoff score of 2.0 for acceptable fidelity.

The EFRS was designed as an observer-rated research scale to be used in reviewing recordings of actual treatment sessions. However, it may similarly be used by an individual clinician to monitor fidelity in his/her own treatment sessions. EMDR consultants may also find the scale helpful when assisting therapists in the development of their EMDR therapy skills.

The EFRS can be used in the evaluation of EMDR treatment sessions for any disorder or presenting problem with which standard EMDR procedural steps and the three-pronged protocol are used in addressing memories of adverse life experiences or current triggers eliciting distress. In addition to being established as an efficacious treatment for PTSD (Watts et al., 2013; WHO, 2013), EMDR standard procedures have been successfully used for symptoms of depression (Gauhar, 2016), panic disorder (Faretta, 2013), chronic pain (De Roos et al., 2010), relational stresses (Reicherzer, 2011), performance anxiety (Maxfield & Melnyk, 2000), and many other problems.

The EFRS is designed to evaluate a therapist's adherence to EMDR therapy's standard eight-phase treatment approach during a single session. It also examines whether a therapist is adequately adhering to the three-pronged protocol, that is, whether the therapist is appropriately addressing relevant past events, present triggers and symptoms, and future behavioral goals. The scale is divided into five subscales, some of which have multiple sections. The five subscales (and sections within each subscale) are as follows:

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I. Introductory Subscale

Phase 1—History-Taking and Treatment Planning Section

Phase 2—Preparation Section

Safe/Calm Place Exercise Section

II. Resource Development and Installation Subscale (optional)

III. Adverse Life Experiences Processing Subscale

Phase 3—Assessment Section

Phase 4—Desensitization Section

Phase 5—Installation Section

Phase 6—Body Scan Section

Phase 7—Closure Section

Phase 8—Reevaluation Section

IV. Future Template Subscale

V. Three-Pronged Protocol Subscale

The EFRS follows. Items and sections identified as “critical elements” are marked with an asterisk. The concept of critical items and their relevance to scoring is addressed in the EFRS manual. The complete EFRS (formatted for use in research), the EFRS manual (containing relevant information about the scale, with instructions for clinicians, raters, and researchers), essential forms, EFRS Excel workbooks with embedded scoring calculators, and a sample scoring workbook may be found and downloaded at <http://emdrresearchfoundation.org/emdr-fidelity-rating-scale>. These materials are available through a Creative Commons Attribution-NonCommercial-NoDerivatives License (<https://creativecommons.org/licenses/by-nc-nd/4.0>).

EMDR FIDELITY RATING SCALE

I. Introductory (Intro) Subscale (History-Taking and Treatment Planning, Preparation, Safe/Calm Place Exercise)

0	1	2	3
No Adherence	Some Adherence But Inadequate	Adherence Acceptable	Adherence Very Good

☐ History-Taking and Treatment Planning (HTP)

- Gathers relevant history (according to framework provided in treatment manual).

0 1 2 3 NA

- *Identifies possible EMDR processing targets (past, present, and future).

0 1 2 3 NA

- Appropriately uses the "affect scan/floatback technique," if information is not obtained from direct questioning, to identify past events related to current disturbance.

0 1 2 3 NA

- Proposes and discusses treatment plan (focused on past adverse life experiences (ALE), current triggers, and future goals) with client.

0 1 2 3 NA

☐ Preparation

(In addition to this subscale, also use Resource Development and Installation [RDI] subscale if RDI is used in a Preparation session)

- Offers a coherent explanation/rationale for EMDR.

0 1 2 3 NA

- Offers clear instructions to client about his/her role.

0 1 2 3 NA

- Provides appropriate physical preparation.

- Arranges chairs for "ships in the night" position. + -
- Introduces and explains use of other bilateral stimulation (BLS) and associated technology if relevant. + -
- Establishes comfortable distance/speed for facilitation of eye movements (EMs) and/or BLS. + -

0 1 2 3 NA

4. Establishes stop signal.

0 1 2 3 NA

5. Establishes metaphor.

0 1 2 3 NA

☐ **Safe/Calm Place Exercise**

1. Offers a coherent explanation/rationale for the Safe/Calm Place exercise.

0 1 2 3 NA

2. Helps client identify an appropriate Safe/Calm Place.

0 1 2 3 NA

3. Asks client to describe a Safe/Calm Place image and identify the emotions and positive sensations experienced in the body. Helps client enhance his/her imagery/experience.

0 1 2 3 NA

4. Asks client to bring to mind the image, encouraging him/her to concentrate on the positive sensations while following the EMs/BLS.

0 1 2 3 NA

5. Introduces one or more short sets of slow EMs/BLS (sets of four to eight back-and-forth or left/right movements) and checks in with client about what he/she is noticing at the end of each set.

0 1 2 3 NA

6. Asks client to identify a cue word or phrase associated with the Safe/Calm Place. Then, asks client to notice the positive feelings and sensations he/she has when focusing on the cue word, encouraging him/her to concentrate on the positive sensations while following the EMs/BLS.

0 1 2 3 NA

7. Instructs client to repeat the procedure on his/her own, bringing up the image and the cue word, and experiencing the positive feelings without EMs/BLS.

0 1 2 3 NA

8. Asks client to bring up a minor disturbing thought, issue, or incident and notice the negative feelings. Then, guides him/her through the Safe/Calm Place Exercise.

0 1 2 3 NA

9. Asks client to bring up a disturbing thought, issue, or incident and do the Safe/Calm Place exercise, this time without assistance.

0 1 2 3 NA

10. If client experiences difficulties with exercise (unable to identify a safe place; dissociative or avoidant response; emergence of negative pictures, emotions, sensations, thoughts), offers appropriate suggestions or interventions.

- a. Assists client in exploration of possible other Safe/Calm Place images. + –
- b. Redirects client back to positive aspects of the Safe/Calm Place image. + –
- c. Introduces container imagery to manage negative material, or shifts to mindfulness or breathing exercise. + –
- d. Uses shorter sets of EMs/BLS or decreases speed of EMs/BLS. + –

0 1 2 3 NA

II. Resource Development and Installation (RDI) Subscale (optional part of the Preparation Phase)

0	1	2	3
No Adherence	Some Adherence But Inadequate	Adherence Acceptable	Adherence Very Good

☐ Resource Development and Installation (RDI)

1. Offers a coherent explanation/rationale for resource development and installation work.

0 1 2 3 NA

2. Asks client to focus on a challenging current life situation (including EMDR treatment itself).

0 1 2 3 NA

3. Asks client to identify what qualities (capacities, strengths, feelings, beliefs, etc.) he/she needs to manage this situation.

0 1 2 3 NA

4. Asks client to identify an image that captures or enhances the desired quality, capacity, feeling and/or strength.

- a. Prompts client to consider previous mastery experiences or images, relational resources, and/or relevant symbolic resources or metaphors. + –
- b. Facilitates the search for resource images if client has difficulty. + –

0 1 2 3 NA

5. Asks client to describe the resource image and identify the emotions and positive sensations experienced in the body. Helps client enhance his/her imagery/experience.

0 1 2 3 NA

6. Asks client to bring to mind the resource image, encouraging him/her to concentrate on the positive sensations while following the EMs/BLS.

0 1 2 3 NA

7. Introduces one or more short sets of slow EMs/BLS (sets of eight to ten back-and-forth or left/right movements) and checks in with client about what he/she is noticing at the end of each set.

0 1 2 3 NA

8. Asks client to identify a cue word or phrase associated with the resource image. Then, asks client to notice the positive feelings and sensations he/she has when focusing on the cue word, encouraging him/her to concentrate on the positive sensations while following the EMs/BLS.

0 1 2 3 NA

9. Continues with sets of EMs/BLS as positive feelings and associations get stronger, and/or stops when the resource is appropriately strengthened.

0 1 2 3 NA

10. Instructs client to repeat the procedure on his/her own, bringing up the image and the cue word and experiencing the positive feelings without EMs/BLS.

0 1 2 3 NA

11. Instructs client to imagine the situation that he/she would like to manage more effectively. Then, asks client to run a movie of his/her desired response, using his/her resource to enhance coping or performance.

0 1 2 3 NA

12. Asks client for feedback and once movie feels positive and strong, introduces several sets of slow EMs/BLS until client feels comfortable and secure with his/her future movie.

0 1 2 3 NA

13. If appropriate, instructs client to imagine a particular challenge that might arise in a future situation. Then, asks client to run a movie of his/her desired response, using his/her resource to enhance coping or performance. Introduces EMs/BLS as in #12.

0 1 2 3 NA

14. Asks client to practice using his/her resource(s) in the actual challenging life situation identified in #2.

0 1 2 3 NA

15. If client experiences difficulties with exercise (unable to identify resource; dissociative or avoidant response; emergence of negative pictures, emotions, sensations, thoughts), offers appropriate suggestions or interventions.

- a. Assists client in exploration of possible other resource associations/images. + –
- b. Redirects client back to positive aspects of resource image. + –
- c. Introduces container imagery to manage negative material, or shifts to mindfulness or breathing exercise. + –
- d. Uses shorter sets of EMs/BLS or omits EMs/BLS. + –

0 1 2 3 NA

**III. Adverse Life Experiences (ALE) Processing Subscale
(Assessment, Desensitization, Installation, Body Scan, Closure, Reevaluation)**

0	1	2	3
No Adherence	Some Adherence But Inadequate	Adherence Acceptable	Adherence Very Good

☐ **Reevaluation (N/A if first reprocessing session)**

1. Obtains feedback on experience since last session (e.g., symptoms; behaviors; reactions to present triggers; new thoughts, insights, or information; dreams; and any new or associated material that may have emerged). Reviews log with client if available.

0 1 2 3 NA

2. Assesses the previous target by asking the client to bring up the memory, incident, or trigger addressed in the previous session (appropriate only if client has had a previous processing session). Asks client what he/she notices and, more specifically, elicits information about the following elements, as appropriate:

- a. Image + –
- b. Emotions + –
- c. SUD (0–10) + –
- d. Body Sensations/Locations + –
- e. Positive Cognition and VOC (1–7) + –
(If positive cognition or future template previously addressed)

0 1 2 3 NA

3. Works with client to select appropriate target for current session.
 - a. If work with previous target remains incomplete (i.e., elevated SUD, low VOC, or body scan not clear), continues with previous target (i.e., proceeding with Phase 4, 5, or 6, as appropriate). + -
 - b. If processing of previous target appears complete (i.e., SUD = 0/1, VOC = 6/7, or ecologically valid ratings; clear body scan), does one of the following, as appropriate: + -
 - i. Shifts to focus on another memory identified in the treatment plan or a feeder memory.
 - ii. Shifts to focus on current trigger or symptom.
 - iii. Shifts to focus on future template.
 - c. If current life crisis or clinical symptoms are significantly destabilizing and it is determined that further processing would not be useful, shifts to Preparation Phase interventions. + -
- 0 1 2 3 NA

☐ ***Assessment (N/A if starting with reevaluation and not assessing a new target)**

1. Helps client to select appropriate target (may be past adverse life experience or present trigger).

0 1 2 3 NA
2. Obtains picture representing the worst aspect of memory or target issue; if unable to obtain a picture, asks client to just think of it and identify the worst part of the experience (e.g., smells, sounds, taste, touch).

0 1 2 3 NA
3. Helps client identify appropriate negative cognition, seeking one that is present tense, generalizable, irrational, and a self-referencing I-statement.

0 1 2 3 NA
4. Helps client identify appropriate positive cognition, seeking one that is possible, generalizable, a self-referencing I-statement, and matching in focus with the negative cognition.

0 1 2 3 NA
5. Obtains accurate VOC (Links positive cognition with picture or issue. Stresses rating of VOC in the PRESENT).

0 1 2 3 NA
6. Helps client identify emotions linked to picture and negative cognition; explores for additional emotion(s) if warranted.

0 1 2 3 NA

7. Obtains SUD level.

0 1 2 3 NA

8. Helps client identify body sensations.

0 1 2 3 NA

9. Follows the standard Assessment sequence, as presented in this subscale.

0 1 2 3 NA

☐ ***Desensitization**

1. Before beginning EMs/BLS, instructs client to bring up picture, negative cognition, and body sensations.

0 1 2 3 NA

2. If resuming processing of an incomplete target memory from previous session, asks client to bring up the image (that represents the worst part of the memory upon reevaluation), identify emotions, rate disturbance (SUD 0–10), and identify location of body sensations. Then, begins EMs/BLS. (Note: The negative cognition is not included when restarting an incomplete target memory.)

0 1 2 3 NA

3. Effectively introduces EMs/BLS and makes sure that client tracks adequately.

0 1 2 3 NA

4. If client's eyes don't follow, clinician makes appropriate response. (If an alternative form of BLS is used, circle NA for this item.)

a. Verbally cues client. + –

b. Slows speed. + –

c. Makes additional finger motions. + –

0 1 2 3 NA

5. Performs initial set of at least 24 back-and-forth or left/right movements (unless client has difficulty tolerating set of 24). If appropriate, subsequently adjusts length of set in response to client need.

0 1 2 3 NA

6. Gives appropriate verbal support during EMs/BLS.

0 1 2 3 NA

7. Stops EMs/BLS gently and instructs client appropriately (some version of "Take a breath. Let it go." and "What comes up for you now?").

0 1 2 3 NA

8. Restarts EMs/BLS at appropriate time without digression, inappropriate discussion, or repetition of client's words. Provides sets of BLS, not continuous BLS, during processing.

0 1 2 3 NA

9. Continues down the same channel with multiple sets until there is apparent resolution. If new material is emerging and/or change is observed, continues to facilitate processing. Does not return to the original incident prematurely.

0 1 2 3 NA

10. When client appears to be at the end of the channel, asks client to "think of the original incident" (not "original picture") and to describe what he/she notices. Resumes processing with sets of EMs/BLS until client appears to be at the end of the next channel.

0 1 2 3 NA

- *11. Handles abreactions (strong emotions, high arousal) appropriately.

a. Keeps eyes moving (or continues alternative form of BLS). + -

b. Provides additional support to maintain dual attention and expresses compassion. + -

c. Does longer sets of EMs/BLS. + -

0 1 2 3 NA

- *12. If material is stuck or looping, therapist intervenes appropriately, using one or more of the following strategies:

a. Increases rate and duration of EMs/BLS. + -

b. Asks client to focus on body sensations. + -

c. Changes directions of EMs. + -

d. Returns to initial incident or memory. + -

e. Changes modalities (e.g., to taps). + -

f. Checks for blocking beliefs or feeder memories. + -

g. Introduces cognitive interweave. + -

0 1 2 3 NA

- *13. Appropriate timing and application of cognitive interweaves (including sensitivity to possible issues of responsibility, safety, and choices).

0 1 2 3 NA

14. Checks SUD score with original incident when appropriate and makes sure SUD = 0 or 1 (or is as low as it can ecologically go) PRIOR to moving on to Installation of Positive Cognition (continues EMs/BLS if SUD is >1 and not deemed ecologically valid).

0 1 2 3 NA

□ ***Installation**

1. Checks for the possibility of a better positive cognition and revises positive cognition if more appropriate one is identified.

0 1 2 3 NA

2. Asks client to think about the incident and the selected positive cognition and checks VOC (1 = completely false and 7 = completely true).

0 1 2 3 NA

3. Does one set of EMs/BLS while client focuses on both positive cognition and incident and then, rechecks VOC.

0 1 2 3 NA

4. Continues with sets of EMs/BLS, checking the VOC as needed until VOC = 7, no longer increases, or is evaluated to be ecologically valid.

0 1 2 3 NA

5. If VOC is not increasing, checks for blocks (i.e., "What prevents it from being a 7?").

0 1 2 3 NA

6. If blocks are identified, addresses them with additional sets of EMs/BLS until VOC = 7 or is evaluated to be ecologically valid. If needed, uses cognitive interweaves to address blocks and/or returns to processing to address emerging associations.

0 1 2 3 NA

7. Once VOC = 7, continues with an additional set or sets of EMs/BLS to further strengthen PC, until its validity and sense of appropriateness appear to reach a maximum level.

0 1 2 3 NA

□ **Body Scan**

1. Upon completion of Installation Phase, facilitates a body scan. Asks client to hold in mind the original incident/experience and the positive cognition while bringing attention to different parts of the body, starting with the head and working downward. Asks client to report on what he/she notices.

0 1 2 3 NA

2. If positive sensations are reported, strengthens and reinforces these sensations with additional set or sets of EMs/BLS.

0 1 2 3 NA

3. If disturbing material, feelings, or sensations emerge during body scan, returns to processing with sets of EMs/BLS. Continues until body scan is clear and no negative sensations are being reported. Note: If clear body scan is not achieved by end of session, contains material or assists with any discomfort.

0 1 2 3 NA

☐ **Closure**

1. Provides appropriate closure.
 - a. Chooses appropriate termination point. + -
 - b. Provides support/normalizes experience. + -
 - c. Adequately debriefs. + -
 - d. Predicts possibility of continued processing between sessions. + -
 - e. Encourages client to call if having difficulties. + -

0 1 2 3 NA

2. If material not completely processed, uses procedure for closing incomplete session (relaxation, visual healing, containment).

0 1 2 3 NA

3. Requests that client journal or maintain a log between sessions.

0 1 2 3 NA

IV. Future Template (FT) Subscale

0	1	2	3
No Adherence	Some Adherence But Inadequate	Adherence Acceptable	Adherence Very Good

☐ **Future Template**

1. Helps client identify a future behavioral goal (related to a challenging recent experience, present trigger, or potential new situation).

0 1 2 3 NA

2. Asks client to imagine coping effectively in a future scene while focusing on a positive cognition (initially without BLS to make sure that client can visualize it).

0 1 2 3 NA

3. Asks client about blocks, anxieties, or fears that arise while imagining the future scene.

0 1 2 3 NA

4. If client encounters blocks, anxieties, or fears, intervenes appropriately.
 - a. Focuses client's attention on disturbance/body sensation and facilitates EMs/BLS. + -
 - b. Problem-solves with client to increase sense of mastery with future template scene; introduces relevant skills, information, or resources. + -
 - c. Introduces cognitive interweave. + -
 - d. Redirects client to past or present targets that may need additional attention and processing. + -

0 1 2 3 NA
5. If there are no apparent blocks and client is able to visualize the future scene with confidence and clarity, asks client to focus on the image, positive belief, and sensations associated with scene and introduces EMs/BLS.

0 1 2 3 NA
6. Facilitates several sets of EMs/BLS until the future template is sufficiently strengthened (check with body scan and VOC).

0 1 2 3 NA
7. Asks client to move from imagining a "scene" to imagining a "movie" of coping in the future, with a beginning, middle, and end.

0 1 2 3 NA
8. If the client encounters blocks, anxieties, or fears, intervenes appropriately (as above in #4).
 - a. Focuses client's attention on disturbance/body sensation and facilitates EMs/BLS. + -
 - b. Problem-solves with client to increase sense of mastery with future template movie; introduces relevant skills, information, or resources. + -
 - c. Introduces cognitive interweave. + -
 - d. Redirects client to past or present targets that may need additional attention and processing. + -

0 1 2 3 NA
9. When the client is able to play the movie from start to finish with a sense of confidence, asks client to play the movie one more time while focusing on the positive cognition; facilitates EMs/BLS while playing entire movie.

0 1 2 3 NA

10. Helps client generate one or more potential challenge situations (unanticipated or undesirable triggers/outcomes). Asks client to run a movie of effectively responding to each challenge situation. Intervenes if client has difficulty (as in #8). Otherwise, installs the positive movie with BLS/EMs (as in #9).

0 1 2 3 NA

V. Three-Pronged Protocol (TPP) Subscale

(Ratings based on review of data from Treatment Plan Tracking [TPT] form available in online manual. Evaluates whether clinician did or did not appropriately identify and process relevant past adverse life experiences, present triggers/symptoms, and future templates associated with a given presenting issue. Completed by rater at end of treatment/research study.)

No	Yes
No Adherence	Adherence Acceptable

☐ **Three-Pronged Protocol**

1. For a specific presenting issue, appropriately identifies target(s) related to past adverse life experiences.

No Yes NA

2. For the same presenting issue, appropriately identifies target(s) related to current triggers or symptoms.

No Yes NA

3. For the same presenting issue, appropriately identifies target(s) related to future behaviors and goals.

No Yes NA

4. For the same presenting issue, processes relevant past adverse life experience(s), using the standard EMDR protocol.

No Yes NA

5. For the same presenting issue, processes relevant associated present trigger(s)/symptom(s), using the standard EMDR protocol.

No Yes NA

6. For the same presenting issue, installs future template(s) relevant to client's desired behaviors and goals.

No Yes NA